

Name:	John Q. Patient	DOB:	01/01/1960
Age:	63	Sex:	Male
Address:	789 Fictional Lane, Meditown, NT 12345	Phone:	(555) 987-6543
MRN:	MRN123456	Account:	ACCT7890
Insurance:	Sample Health Plan	Emergency Contact:	Jane R. Patient - (555) 555-1234
Encounter Date:	09/15/2023	Check-in Time:	08:00 AM
Attending Gastroenterologist:	Dr. John A. Doe, MD	Referring Provider:	Dr. Emily Q. Referrer, DO

Clinical Information

Chief Complaint / Indication: Evaluation of anemia and dysphagia.

PMH: GERD, HTN, Type 2 DM.

PSH: Appendectomy.

FHX: Heavily positive for gastric cancer in father.

SHX: Former smoker, occasional alcohol.

Allergies: Penicillin (rash).

Problem List: GERD, hypertension, type 2 diabetes mellitus.

Medication	Dose	Frequency
Omeprazole	20 mg	Daily
Lisinopril	10 mg	Daily
Metformin	500 mg	BID
Aspirin	81 mg	Daily
Atorvastatin	40 mg	Daily

Pre-Procedure Vitals + ASA Classification

BP: 130/85 mmHg, HR: 75 bpm, SpO2: 98%, ASA II.

Informed Consent

The risks, benefits, and alternatives of the procedure were discussed with the patient, who understood and consented to proceed.

Airway: Mallampati class II, adequate mouth opening.

Cardio/Pulm: Clear to auscultation, regular rhythm, no murmur.

NPO since: Midnight before procedure, last intake - water at 06:00 AM.

Planned Sedation

Monitored Anesthesia Care with Propofol 200 mg, Midazolam 2 mg, Fentanyl 100 mcg.

Anesthesia Team

Anesthesiologist: Dr. Anne N. Easthesie, MD

CRNA: Joe P. Nurse, CRNA

Procedure Timing

Anesthesia Start: 09:15 AM | Procedure Start (Scope In): 09:23 AM

Procedure End (Scope Out): 09:45 AM | Anesthesia End: 09:50 AM

Recovery Start: 09:55 AM | Recovery End: 10:45 AM

Monitoring Trends

Time	BP	HR	SpO2
09:23 AM	128/82	72	98%
09:35 AM	125/80	70	97%
09:45 AM	122/79	68	98%

Complications

Brief desaturation to 92%, resolved with jaw thrust and O2 adjustment.

Post-Anesthesia Evaluation

Aldrete Score: 9/10. Disposition: Home with responsible adult escort.

Endoscope: Model X2000, peroral insertion. Patient was in the left lateral decubitus position with a bite block in place.

Procedure Narrative

The endoscope was gently advanced through the oropharynx and passed through the upper esophageal sphincter without difficulty. In the esophagus, mucosa appeared normal with no evident rings or strictures; the Z-line was identified at 40 cm from the incisors.

In the stomach, moderate insufflation was used to aid visualization. The fundus, body, and antrum were carefully inspected, revealing mild erythema in the antrum without erosions or ulcers. The pylorus was open and traversed easily into the duodenum.

The duodenal bulb appeared normal, and in the second portion, the villi were intact, and the mucosa demonstrated no focal lesions or inflammation. Retroflexion in the stomach allowed further assessment of the cardia and fundus, noting no abnormal findings.

Findings

- Esophagus: Normal mucosa, Z-line at 40 cm.
- Stomach: Mild antral erythema, no ulcers.
- Duodenum: Normal appearance, intact villi.

Impression

Mild antral gastritis.

Diagnosis

Diagnosed with mild gastritis; GERD evaluation consistent with known history.

Interventions

Biopsies were taken from the antrum using cold forceps. Three samples were collected for pathology. Hemostasis was achieved post-biopsy with no active bleeding observed.

Endoscopy Team

Endoscopist: Dr. John A. Doe, MD | Assistant: Jane S. Helper | RN Circulator: Mary N. Nurse | Endoscopy Tech: Mark T. Tech

Container	Site	Fragments	Method	Fixative
A	Antrum	3	Cold Forceps	Formalin

Collection/ Lab Details

Collection Time: 09:45 AM | Sent to Lab: 10:00 AM

Lab Name: Synergy Labs | Accession Number: LAB-TST001234

Pathology Report

Gross Description: Three tan mucosal fragments ranging from 2-4 mm.

Microscopic Description: Gastric mucosa with mild chronic inflammation, no H. pylori identified.

Final Diagnosis: Mild chronic gastritis, negative for H. pylori.

Pathologist Signature

Dr. Path O. Logist, MD | Report Date/Time: 09/16/2023, 12:00 PM

Patient experienced minimal pain and no nausea. Tolerated sips of water and had stable vital signs throughout recovery.

Diet / Activity Instructions

- Resume regular diet as tolerated.
- Avoid spicy and acidic foods for 24 hours.
- Rest for the remainder of the day; resume normal activity tomorrow.

Warning Signs

Contact office immediately if experiencing severe abdominal pain, persistent vomiting, or bleeding.

Follow-up Plan

Follow up in Gastroenterology Clinic within 2 weeks.

Medication Plan

Continue current medications. Start oral PPI twice daily for gastritis management. Avoid NSAIDs.

Results Communication

Pathology results to be discussed at follow-up appointment.

Patient Understanding & Escort

The patient verbalized understanding of discharge instructions and left with a responsible adult escort.

RN Discharge Educator

Nurse: Emily T. Guide, RN | Discharge Time: 10:50 AM

Provider Signature

Dr. John A. Doe, MD | Date/Time: 09/15/2023, 10:55 AM

Electronically signed by Dr. John A. Doe, MD